

An Older Autistic Adult in Your Clinic

For family doctors & health professionals — caring for an older autistic adult (roughly ages 60+).

The one idea

Autism, through **monotropism**, means an older adult's attention has always locked into a few deep, intense "tunnels." This is the **least-researched part of the autistic lifespan**, and outcomes are concerning: autistic older adults have **higher rates of most health conditions, low quality of life, and limited social participation**. Many were never identified — the "lost generation" — and arrive via lifelong social/sensory differences, burnout, or crisis. Lifelong interests and routines are anchors of wellbeing; hearing and vision changes compound baseline sensory differences.

What you may see — and what's really going on

You may see...	What's often happening
Avoids care, misses appointments, "doesn't like coming in"	Lifelong negative experiences + sensory/communication barriers — not non-compliance
Decline in skills, exhaustion, withdrawal	Possibly autistic burnout accumulated over decades — often misread as "ageing"
Slow, unusual responses; seems confused	Often processing time + lifelong autistic difference — not necessarily dementia
Strong routines, objects, interests; resists change	These are protective anchors for identity and cognition
Doesn't report pain or symptoms	Interoception differences; "not complaining" ≠ "not suffering"

Try this

- **Offer the AASPIRE AHAT**; allow a trusted supporter; provide **written, literal** information and ample processing time.
- **Lower sensory load** and account for hearing/vision loss compounding baseline differences.
- **Protect lifelong interests and routines**; adapt activities to changing ability rather than removing them.
- **Distinguish autistic difference from cognitive decline carefully** — the autism + dementia evidence is thin; assess, don't assume.
- **Document effective accommodations** so they are automatic next time.

Avoid this

- Writing off lifelong social/sensory differences as “just ageing,” or burnout as depression.
- Disrupting routines or removing meaningful objects in hospital/care settings.
- Assuming cognitive incapacity from a slow or atypical response — use supported decision-making.

When to get more support

Screen for **autistic burnout** (often decades in the making) and treat it as distinct from depression. Watch for the **common co-occurrences** — mood disorders, sleep, GI and cardiac conditions — which are more frequent in this group. Address **social isolation** (a major risk) and encourage gentle, tailored activity. Be transparent about the **limits of the evidence** in old age, and plan advance care with clear, literal communication.

If the person is a woman

Older autistic women are an especially under-identified group, having masked for a lifetime. A history of “anxiety,” misdiagnoses, or burnout alongside lifelong social/sensory difference warrants serious consideration of autism.

About this guide

*AI-assisted, derived from this project's research drafts — the **Monotropism** brief and the **Lifespan Practitioner & Health-Care guide** (Part 3.5 & 4). Uses identity-first language. **Informational — not medical advice or a diagnostic tool**. Note: old-age autism evidence is thin; treat as best-practice guidance, and be honest about uncertainty.*

Key sources. Murray, Lesser & Lawson (2005); Dwyer (2025); Klein et al. (2024, aging & autism review); Raymaker et al. (2020); Kelly Mahler (2024); AASPIRE AHAT; OAR (aging on the spectrum). Full citations are in the source drafts.